NCCP 00361 · VERSION 4

Fulvestrant · injections every 4 weeks

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days.

> Clinician supportive-care discussion draft. This is not an issued prescription.

Minimal emetogenic risk

No routine prophylactic antiemetic, corticosteroid premedication or G-CSF order is populated for this endocrine monotherapy. Review persistent nausea or other symptoms and prescribe individually if needed.

Medicine-specific support

- Discuss injection discomfort, flushes, joint or muscle symptoms, fatigue and nausea. Ask early for symptom help or practical support attending appointments.
- Fulvestrant alone does not require routine steroid premedication, injections to raise white cells or a standard anti-sickness course. A combination medicine can add its own supportive-care needs.
- Tell staff about blood thinners, unusual bruising or bleeding before an injection. Severe buttock pain, new numbness, weakness or pain down the leg needs prompt assessment.
- Confirm the extra loading visit and later appointments, who books required blood tests and scans, and who reviews the results. Tell the laboratory team about fulvestrant if an oestrogen blood test is ordered.
- Discuss the treatment aim, injection discomfort and possible nerve injury, bleeding or bruising, liver effects, flushes, joint symptoms and fatigue. Liver impairment or an existing modified dose requires an agreed prescriber and pharmacy plan. Where relevant, use effective contraception during treatment and for two years after the final dose under the EMA product information. Pregnancy and breastfeeding are excluded. Tell staff about blood thinners; do not stop them yourself.
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Individual plan

Prescriber / date: _____ Pharmacy review: _____

Verify allergies, interactions, relevant organ function, treatment setting and the exact medicine before prescribing.

NCCP 00361 v4 · Monitoring and treatment map